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Name: نادية رفاعي محمد فراح National ID: 27812181401661	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



القرار ميكت
من ١٤٧
قرار قيسارا





Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m.753

Female	النوع	نادية رفاعي محمد	اسم المريض
بذها	العنوان	46 Y	المسن
17/04/2024 13:09	تاريخ استلام العينة		تاريخ اخذ العينة
29/04/2024 11:36	تاريخ التقرير	مستشفى بنها التعليمي	طالب التحليل
		مستشفى بنها التعليمي	غاية

Primary Sample :

Paraffin Block

Test :

HER2 by SISH

Comment :

Technique :

Sample received was stained for HER2 gene and chromosome 17 using INFORM HER2 Dual Probe Cocktail technique in Benchmark GX / XT (VENTANA).

Result :

Her 2 / chr 17 is more than 2 (core biopsy)

Referral lab :

Comment :

Conclusion :

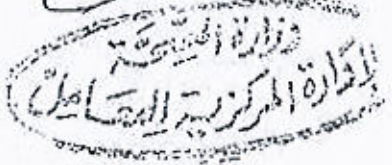
Her 2 Status by SISH (Dual Ish Technique): Positive (amplified Her 2 / chr 17 > 2)

Pathologist:

د/أيهاب رمزي

Lab Director:

د/ترزا ماهر



9 El-Sheikh Rehan Street, Abeidcen, Cairo
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قسم التحليلات المركزية للمعمل

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وتم الاحتفاظ بالنسب العينة لمدة 30 يوم من تاريخ استلامها

Patient Name : الأستاذ / نادية رفاعى محمد فراج
Patient No. : 27812181401661
Path No. : 4261/24
Sex : Female
Age : 45 Y
Referring Doc :

Request No. : 13004119
Request Date : 04/11/2024
Verification Date : 11/11/2024
Financial Category : نفقة الدولة - نورام دار السلام
Department : Out Patient
Registered by : marwa mohamed yous

Pathology Report

Clinical Data:
Right breast cancer post NACT.

Nature of the specimen:
Right Simple mastectomy and sentenil lymph node biopsy specimens , collected at
4/11/2024.

Gross Examination:

- Two specimens were received :
- Sentenil lymph node biopsy specimen :Fibrofatty tissue peice measured 5x4 cm enclosing 8 nodules measured 1.5x1x1 cm.
- Simple mastectomy specimen measured 26x18x5 cm with covering skin ellipse measured 24x16 cm
- Tumor extent
- o Number: An ill defined greyish white and pinkish rubbery to firm gritty tumor bed site.
- o Size:7x4x2 cm
- Distance from margins:3 cm from deep margin and 2 cm from skin.
- Frozen section:
- Sentenil lymph nodes :Positive (3/7).

Microscopic examination:

- Histological type:Extensive ductal carcinoma insitu with foci of microinvasive carcinoma.
- Cellularity: 60%.
- Angioinvasion :Positive.
- Skin involvement :Free nipple and areola.
- Lymph node involvement: Positive (4/8) with extracapsular extention, one more node was - found in paraffin examination.
- Resection Margins: Free deep margin .

Verified by :
dr.esraa magdy

Printing Date : 24/11/2024 10:48:27

Page 1 of 2

Printing By : Mrs.Mayada Hegazy

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / نادية رفاعي محمد فراج :	Request No. : 13004119
Patient No. : 27812181401661	Request Date : 04/11/2024
Path No. : 4261/24	Verification Date : 11/11/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 45 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Additional pathologic findings: Extensive microcalcifications.
Histologic response to therapy: Pathologic partial response.

Diagnosis:

Right breast cancer post NACT , simple mastectomy and sentinel lymph node biopsy
specimens:

Extensive ductal carcinoma insitu with foci of microinvasive carcinoma.

Free nipple and areola.

Free deep margin .

Lymph node involvement: Positive (4/8) with extracapsular extension..

Pathologic partial response.

Stage : ypT1mi , ypN2.

RCB:3.408, class:III.

Prof. Gamal Abdelatty

Dr. Duaa Muhammed

Verified by :

Dr. Esraa Magdy

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Page 2 of 2

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1990

and Organization for Teaching
and Institution
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المستشفيات والمعاهد التعليمية

مؤسسة محمد بن سعود الطبية

قسم الباثولوجي

الرقم : 598
2024 / 02 / 28
تاريخ الإصدار : 2024

مؤسسة : نادية دفاعي محمد قراج

من :

تاريخ : 2024/02/18

E OF SPECIMEN :

Right breast lesion tru- cut biopsy:

Three gray white tissue cores measured 1.2cm in long, totally submitted.

SCOPY :

Sections examined revealed invasive ductal carcinoma (NST). Tumor forms sheets, nests, cords of anaplastic ductal cells among desmoplastic stroma. Tumor cells showed moderate anaplasia, moderate tendency to form tubules, with occasional mitosis. Ductal carcinoma insitu 60% (solid type) could be seen.

Q&A :

Right breast lesion tru- cut biopsy; INVASIVE DUCT CARCINOMA (NST) PROVISIONAL GRADE I DUCTAL CARCINOMA INSITU 60% (SOLID TYPE).

RENDATION :

مؤسسة



قسم الباثولوجي

السن : 46 سنة

إسم المريض : نادية رفاعي محمد قراج

(2024) (598 - 2024)

محول من :

تاريخ الإستلام : 2024/03/27

تاريخ الحصول : 2024/03/21

NATURE OF SPECIMEN :

One paraffin block .

GROSS :

One paraffin block: sections immunostained for ER, PR, Her-2/neu , ki67, using supersensitive monoclonal antibodies with En- Vision TM+ (HRP) detection system employing peroxidase after antigen retrieval and blocking internal peroxidase and using DAB as chromogen using BenchMark GX automated immunostainer.

MICROSCOPY :

Estrogen receptors: Strong nuclear staining in 70% of tumor cells .
Progesterone receptors: Moderate nuclear staining in 30% of tumor cells.
Her-2/neu: Moderate membranous staining in >10% of tumor cells.
Ki67: Positive nuclear staining in 20% of tumor cells.

DIAGNOSIS :

POSITIVE ESTROGEN RECEPTORS 8/8.
POSITIVE PROGESTERONE RECEPTORS 5/8.
HER-2/neu, SCORE 2+ (EQUIVOCAL)
KI67: POSITIVE NUCLEAR STAINING IN 20% OF TUMOR CELLS.

RECOMENDATION :

د/رشا فاروق



MDT form

A-Patient ID:

1- Name : نادية رفاعي محمد قراج

Sex:

3- Medical number:
Female

83204105

Old Med :

Did not

1353-2024

2-ID number:

Patient Age : 46 Y



Family history	Family history -ve	Phone	Phone 01143902983	BMI	NMI	REASON OF MDT	REASON OF MDT POST OPERATIVE
marital status	Marital Status MARRIED	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date of MDT
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status MENOPAUSAL	Initial STAGE	Initial STAGE	05/12/2024	☐

Clinical data

Breast Imaging :

solid lesion occupier UIQ from 9 o'clock more than 8cm
Newly developed deep lesion UIQ 2 * 1.2cm

Mets Workup :

FREE METASTATIC WORKUP

BX IDC	ER	PR	Her 2 Neu	SISH	Ki67
ER COMPONENT +ve	PR COMPONENT +ve	Her 2 Neu COMPONENT +ve	SISH COMPONENT	Ki67 20	

T 3 MULTI CENTRIC	N 3	M 0
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Mammography

PATIENT HISTORY

pt received AC x4+4 taxotere+3 HP with v.good clinical response
RT SM+SLND LN 4/8 +extracapsular extension
partial pathological response
extensive DCIS+ microinvasion carcinoma YPTMIC, N2
free nipple , areola, deep margin

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

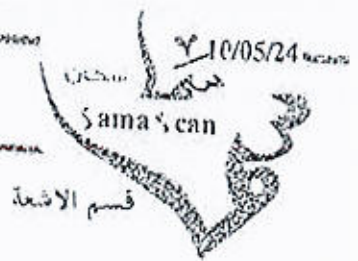
- 1- TDM1X12 CYCLES
- 2- PORT
- 3- ZOLADEX FENARA

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY [Signature]	RADIOLOGY	RADIOLOGY [Signature]
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



AMM 05/05/24



▽ Echo-Doppler study ▽

NAME: Nadia Refaey Mohammed

Age: 46 years

REF. BY: Prof. Dr.

Data: 2024-05-10

Left ventricle			Left Atrium	3.4	(1.9 - 4.0 cm)
EDD :	5.0	(3.5 - 5.6 cm)	Aortic Root :	3.2	(2.0 - 3.7 cm)
ESD :	2.8		AV. Opening		(1.6 - 2.6 cm)
Shortening %	42%	(25 % - 40 %)	Rt. Ventricle :		(0.6 - 2.3 cm)
EF :	73%	(>53%)			
SWT	1.0	(0.7 - 1.1 cm)			
PWT	1.2	(0.7 - 1.1 cm)			

Interventricular septum: Intact	Mitral valve: Normal
Interatrial septum: Intact	Aortic valve: Normal
Thrombi: Absent	Pulmonic valve: Normal
Pericardium: No effusion	Tricuspid valve: Normal

M-Mode, 2-D Doppler & CFM revealed:

- Normal L.V dimensions with good systolic function (E.F 73% by M-Mode).
- No Resting segmental wall motion abnormalities.
- Intact interatrial & interventricular septum.
- Normal mitral valve structures with mild MR.
- Normal aortic valve structures with normal function.
- Normal right side of the heart, mild TR. No pulmonary HTN.
- Normal pulmonary valve structures & flow.
- Normal pericardium.
- No masses or thrombi by T.T.E.

Conclusion:

❖ Mild MR.

Signature
Dr. Hussein (Awad (msc)

بناها - النجدة ت: ٠١٢٢٢٢٦٢٦٢ | بنها - العادلي ت: ٠١٢٢٢٥١٢٢٥ | طوخ ت: ٠١٢٢٤٦٩٢٠٢ | قليوب ت: ٠٢٤٢١٠٢٢٢٢ | زفتى ت: ٠٤٠٤٧٢٩٢٢١
طنطا - ش. البحر ت: ٠٤٠٣٢٥٨٤٢٤ | طنطا - ش. المديرية ت: ٠٤٠٣٣٤٩٩٩٠ | حلمية الزيتون ت: ٠٢٢٦٢٥٨٢٩٥ | الخط الساخن ١٧٠٩٥



Patient Name: Nadia Refaie Mohamed Farraag	Date : 03/11/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 46 Year(s)
Medical ID: 1353/2024	Pat. ID: 93846

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast pathologically proven malignant mass on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Notable right breast shows upper quadrants segmental pleomorphic microcalcification involving an area about 1.1 x 9.8 cm and 2.4 cm away from the nipple.
- Complementary ultrasound revealed regression of right breast small hypoechoic mass lesion, seen at 1-2 o'clock measuring 9 x 4 mm compared to 20 x 12 mm previously. No other masses could be detected.
- No left breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the left breast.
- Nonspecific axillary LN, bilaterally
- Free infra, and supra-clavicular regions.

Opinion:

- Status follow up of right breast malignant process showing above described changes (BIRADS 6).
- Benign findings left breast (BIRADS 2).

Yours truly

Signature Dr. Wafaa Ali

03/11/2024 11:53 AM

Signature Dr. Wafaa Ali

03/11/2024 11:54 AM

Patient Name: Nadia Refaie Mohamed Farraag	Date : 09/09/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 46 Year(s)
Medical ID: 1353/2024	Pat. ID: 93846

Technique: CT scan of the chest , abdomen and pelvis obtained pre and post IV contrast administration. Oral contrast not given.

Findings:

**** Chest :-**

- * A small calcified nodule measures 3x3mm is seen in right lower lobe and multiple small subpleural nodules
- * No nodular or cavitating pulmonary lesions in left lung .
- * Clear both costophrenic recesses
- * No enlarged hilar or other mediastinal lymphadenopathy or other mediastinal masses
- * Few enlarged peri-tracheal and prevascular lymph nodes .
- * Average cardiac size . No pericardial sac collection .

**** ABDOMEN & PELVIS :-**

- * Average size liver showing homogenous density . No C.T. evidence of hepatic focal lesions and
- No dilated intra or extra-hepatic biliary ducts .
- * Patent main trunk and both branches of the portal vein .
- * Normal C.T. appearance of the spleen , gall bladder , pancreas , adrenal glands , pelvic organs , aorta and I.V.C.
- * A well defined rounded soft tissue measures 14x14mm is seen related to splenic hilum , likely splenule .
- * Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- * No retroperitoneal lymphadenopathy .
- * No ascites .

**** EXAMINED SKELETON :-**

- * Multiple sclerotic lesions are seen in D11,12 and L 1,2,3,5 vertebrae and S1 .

**** OPINION :**

- * A case of right MRM showing ;
- Multiple dorsal & lumbar spine deposits .
- No lung or hepatic deposits .

Signature Reeham.Khader

15/09/2025 1:44 AM

Signature Reeham.Khader

15/09/2025 1:45 AM